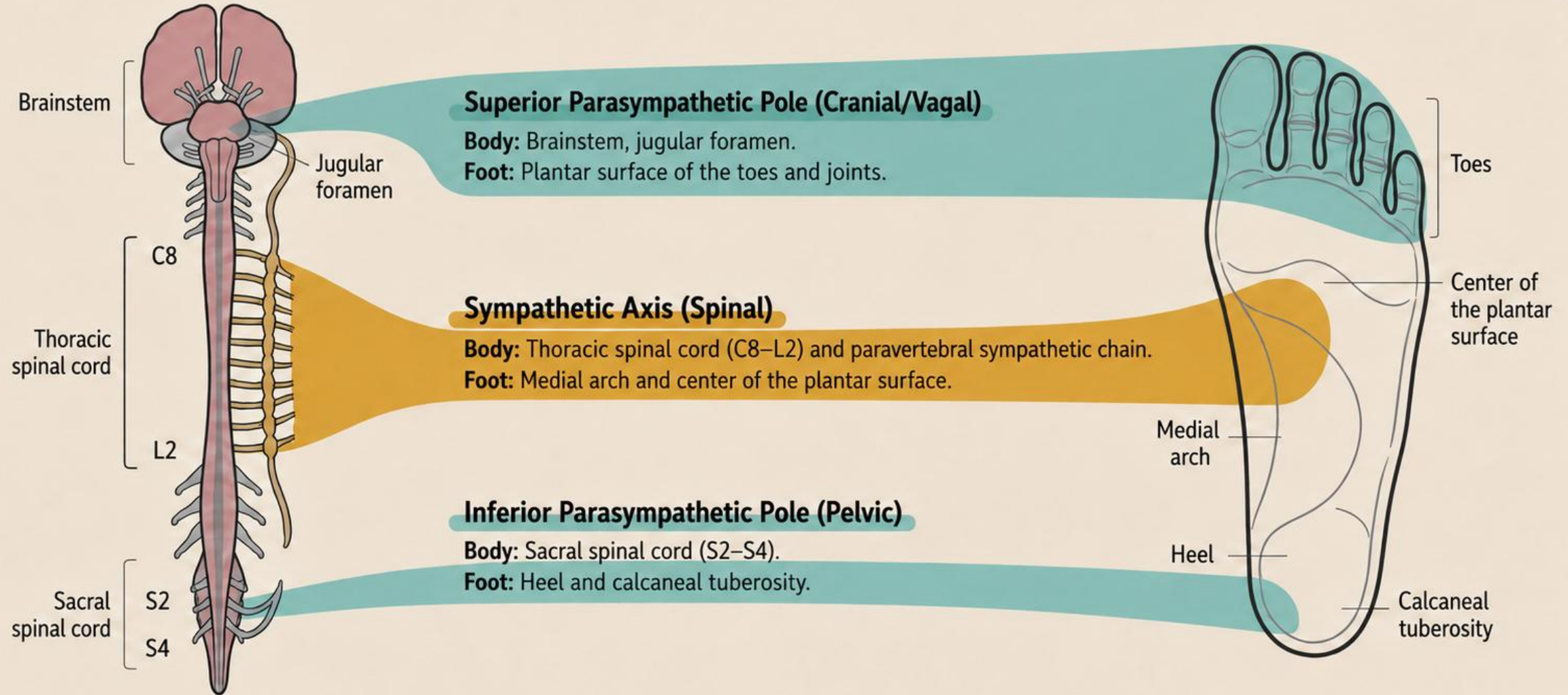


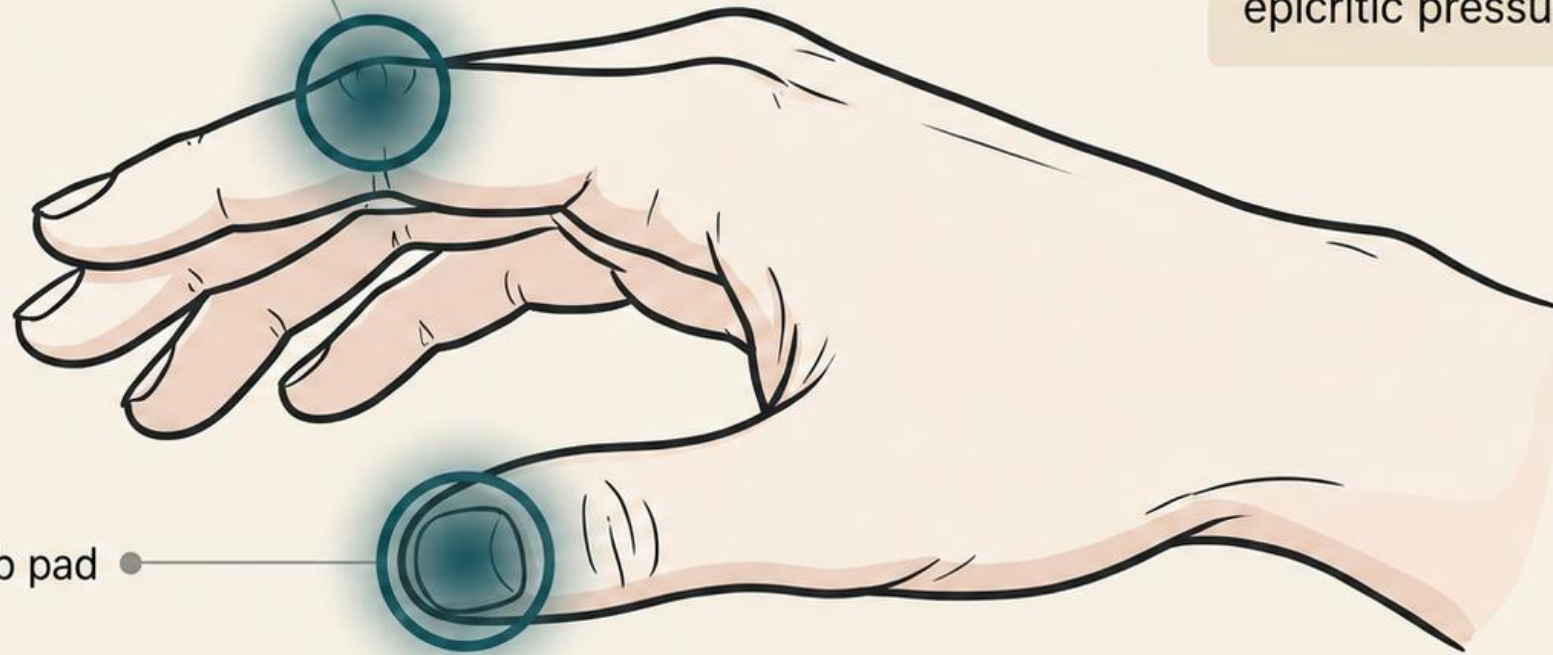
Understanding the Autonomic Nervous System Through Its Major Reflex Zones on Foot



The Precision of the Therapeutic Gesture

Proximal interphalangeal
joint of the index finger

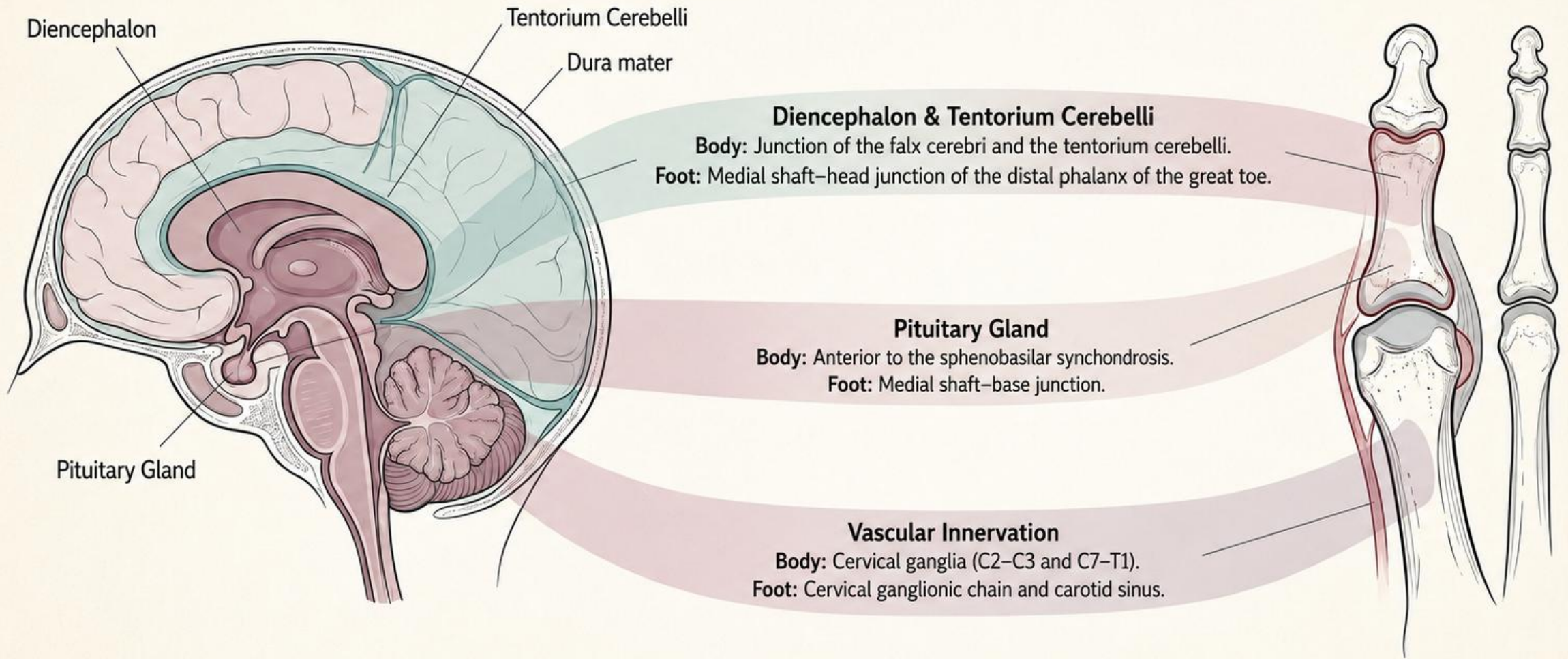
Thumb pad



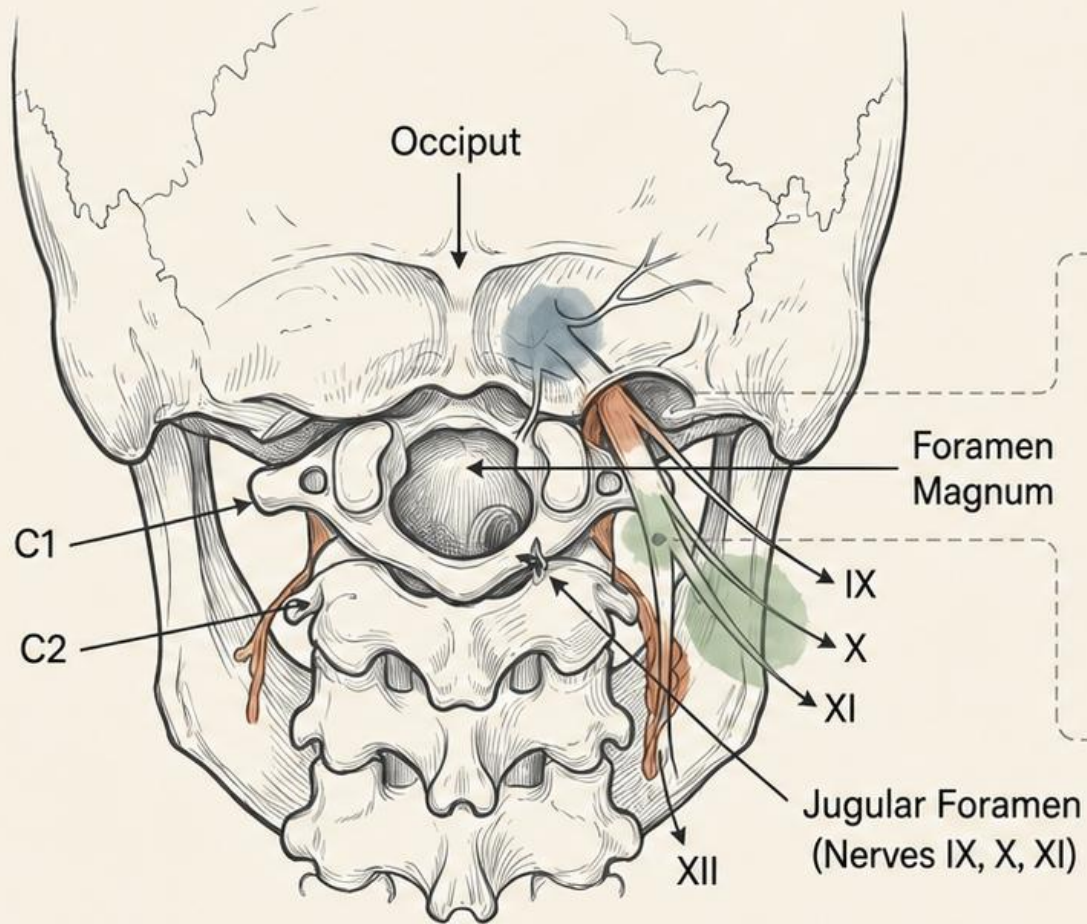
An exclusively manual approach,
without the use of oil or cream,
based on the precision of
epicritic pressure.

Forceful pressure and kneading are contraindicated. The greater the pressure, the more difficult it becomes to identify the bony landmarks and reflex zones.

ROP Correspondence: Diencephalon & Dura Mater



Clinical Focus: The Upper Level



Target Structures:

- Craniocervical junction (Occiput–C1–C2) and foramen magnum.
- Jugular foramen (passage of cranial nerves IX, X, XI).
- Trigeminal nerve (V) and hypoglossal nerve (XII).

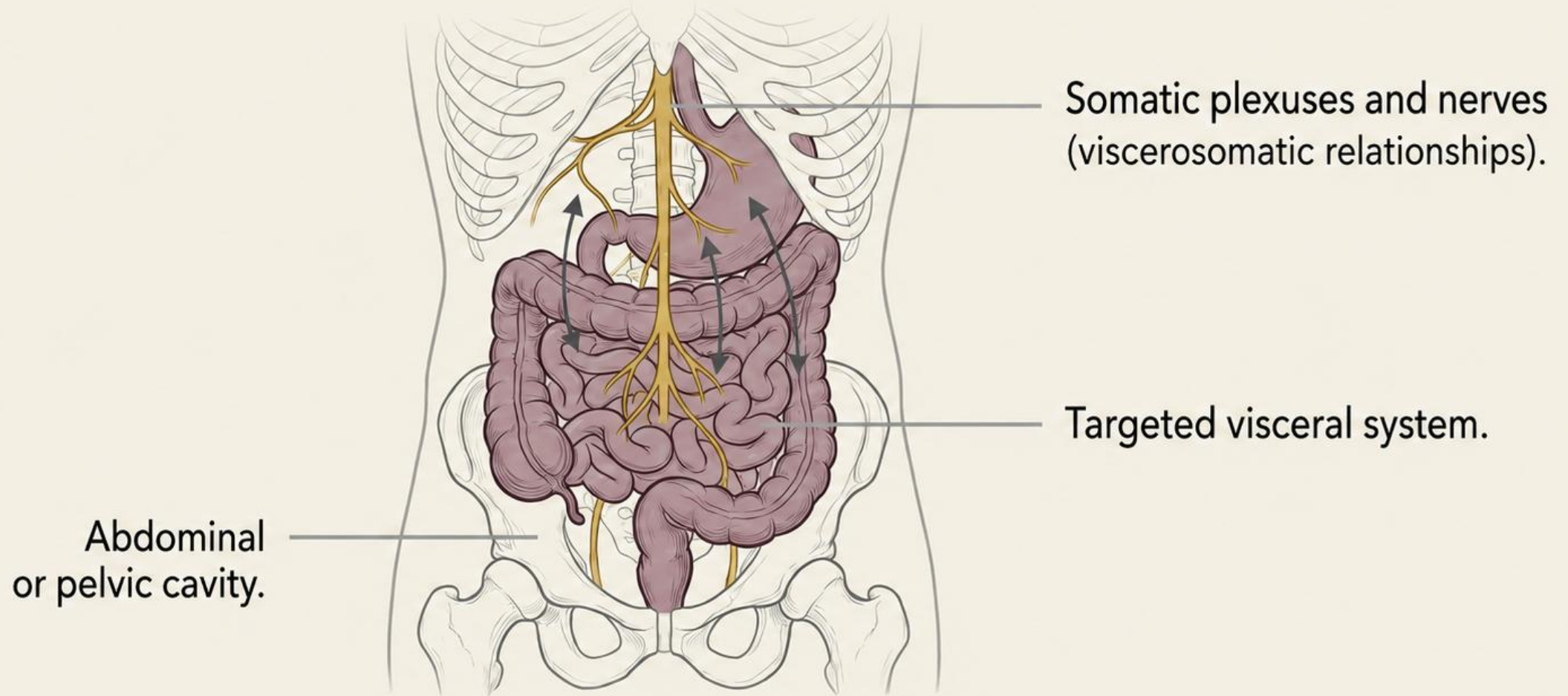
Key Techniques:

- Sphenobasilar synchondrosis (SBS) rebalancing - S2.
- Compression of the 4th ventricle.

Therapeutic Objective: Stimulate the vagus nerve anew to establish a 'neuroception of safety' and regulate the entire autonomic cascade.

Target II: Locoregional Syndrome

Treatment of the loops linked to the symptomatic region.



The objective is to release local fixations in the affected cavity in order to restore tissue mobility and break the neurovegetative loop maintained by the dysfunction.

Vascularization of the Lesser Curvature

Celiac trunk
(T12)

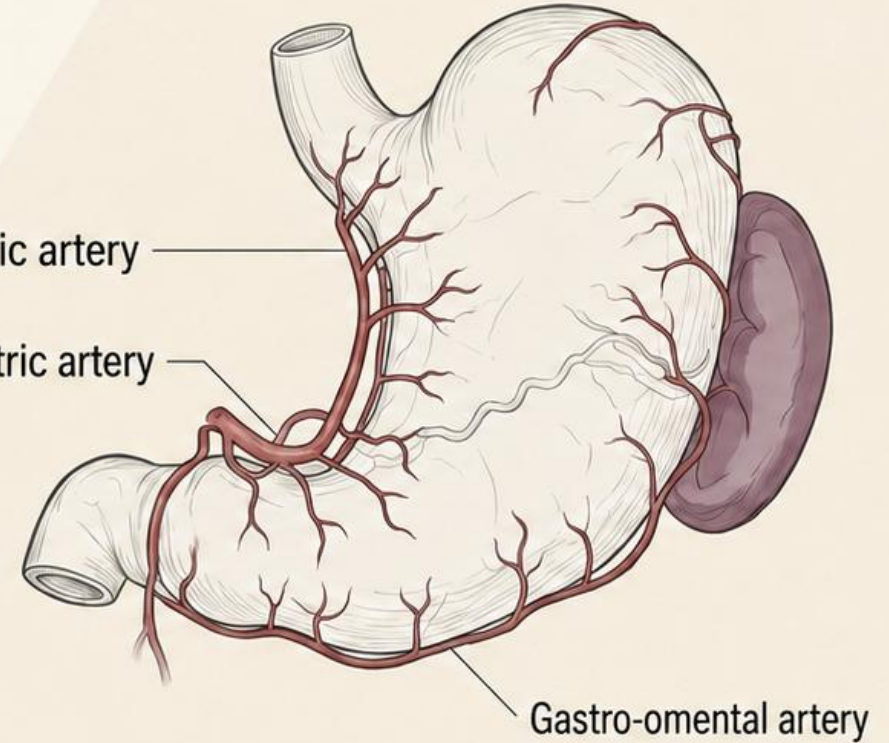


Splenic artery,
Left gastric artery,
Right gastric artery.

Left gastric artery

Right gastric artery

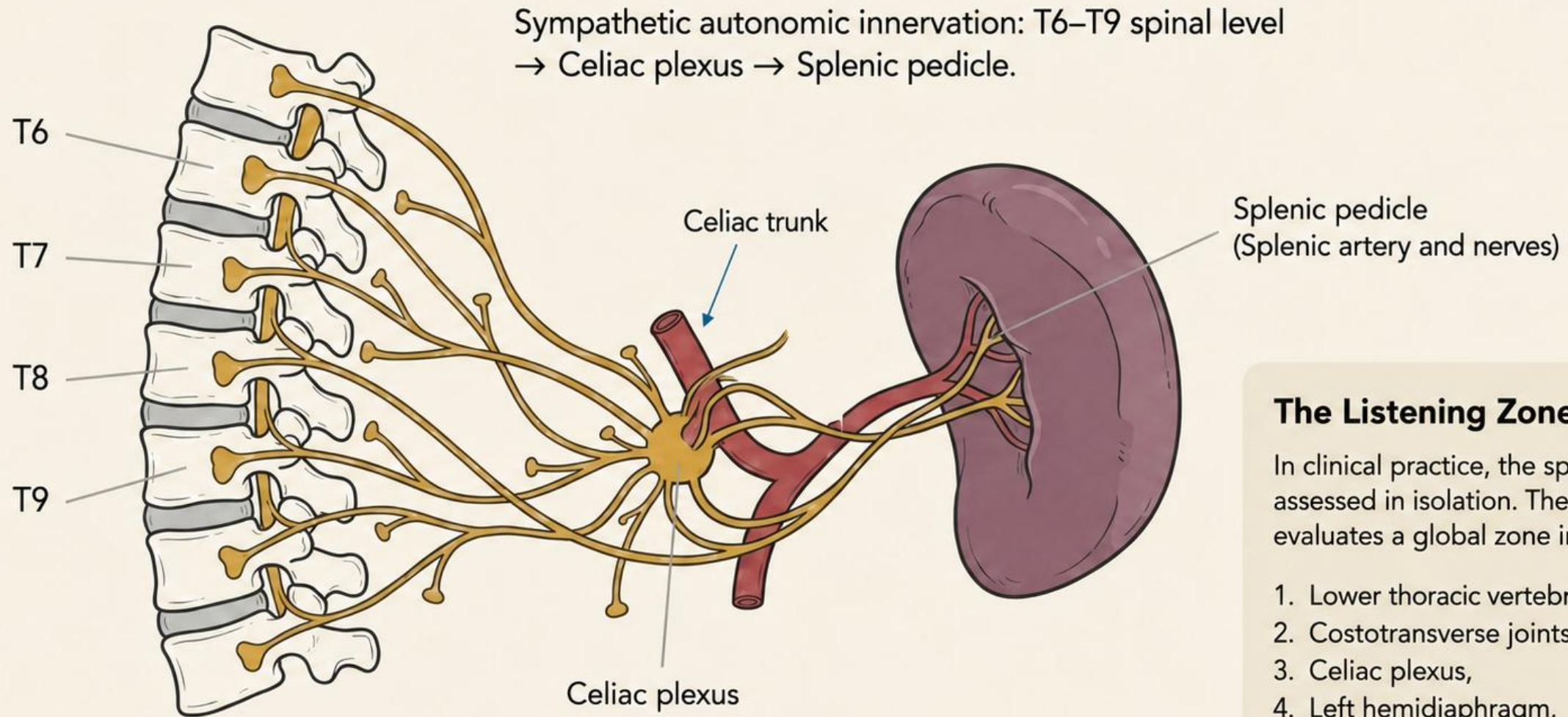
Gastro-omental artery



Practical Relevance in ROP

The lesser curvature requires priority attention. Deeply situated and closely apposed to the spine (T10-L1), it contains a rich vascular supply and parasympathetic innervation. It is the preferred site of mechanical fixations and gastric ulcerations.

Innervation and "Listening Zone" ROP



The Listening Zone (ROP)

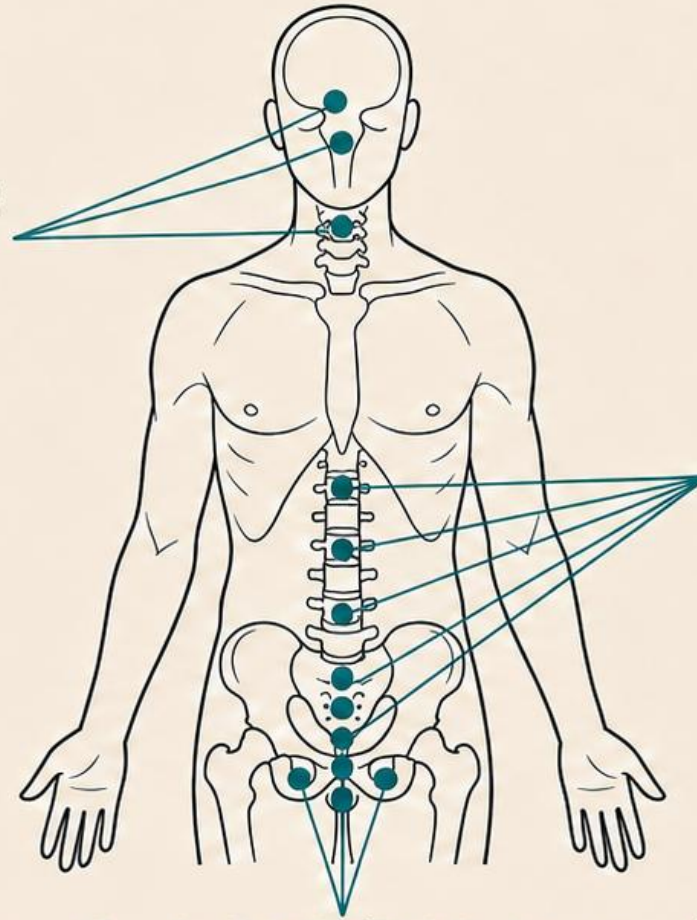
In clinical practice, the spleen is not assessed in isolation. The practitioner evaluates a global zone incorporating:

1. Lower thoracic vertebrae,
2. Costotransverse joints,
3. Celiac plexus,
4. Left hemidiaphragm,
5. Splenic pedicle

Mapping the ROP therapeutic approach

Cranio-Sacral Axis (Limbic Regulation):

- Sphenobasilar synchondrosis.
- Brainstem and hypothalamus.
- **Indication:** Enuresis, emotional dominance, fears.



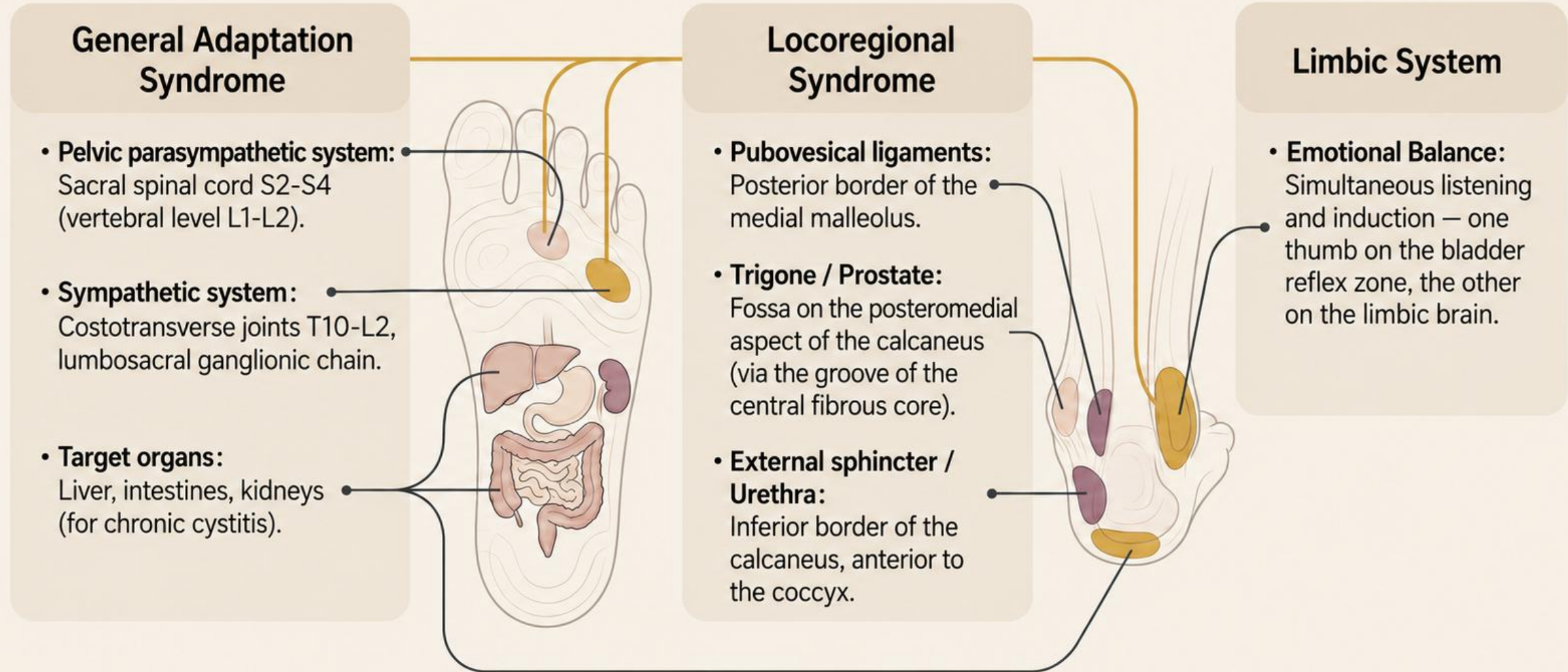
Neurovegetative Axis (Innervation):

- Sympathetic: T11-L2 vertebrae.
- Parasympathetic: S2-S4 sacrum and coccyx.
- **Indication:** Restoration of the neurologic balance between bladder filling and micturition.

Biomechanical Axis (Pelvic Structure):

- Pubic symphysis and obturator fossae.
- Central fibrous perineal body and pudendal plexus.
- **Indication:** Rebalance tension in the urethral fasciae (SUI) and release the pubovesical ligaments.

The ROP sequential approach



Reflex Zone Cartography

General Adaptation Syndrome

- Spine and costotransverse joints (sympathetic ganglion chain T6-T9).
- Celiac plexus.

Loco-Regional Syndrome

- Related organs: Stomach, pancreas, left kidney, root of the mesocolon.
- Spleen zone: Located at the splenic flexure of the colon (medial and superior to the styloid process of the left 5th metatarsal), beneath the diaphragmatic dome.
- Liver: Work on the left lobe and the entire liver to improve splenic vein circulation.

ROP Complement

- Limbic Brain-Spleen Balancing: Listening-induction technique (one thumb on the spleen, the other on the limbic brain).

